

Bridging Two Gaps: A Roadmap for Collaboration Between Occupational Physicians and General Practitioners

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SUMMARY

Occupational physicians (OPs) and general practitioners (GPs) manage overlapping aspects of the same patient's health yet rarely communicate with one another. By assigning OPs a role in promoting cancer screening, Law 198/2025 has created in Italy a regulatory link between occupational surveillance and public health prevention—a link that cannot function without a structured exchange of data between the two professional roles. This commentary argues that the new regulatory framework, together with Legislative Decree 135/2024 on carcinogens, mutagens, and substances toxic to reproduction, and Legislative Decree 29/2024 on active aging, offers an unprecedented—yet narrow—window of opportunity to build formal collaboration between OPs and GPs in Italy. Another argument concerns the growing presence of foreign-born workers, who may carry undiagnosed endemic conditions and face language barriers that make OP-GP coordination essential for safe fitness-for-work assessments. Drawing on documented challenges from similar initiatives in the Netherlands, Germany, France, and Finland, we identify the main structural obstacles in the Italian context and propose a three-tiered roadmap for implementation, progressing from immediate consensus-based models to the long-term integration of occupational health data into the Electronic Health Record 2.0.

1. A LEGISLATIVE SHIFT

Until recently, the statutory obligations of Italian occupational health physicians (OPs) under Legislative Decree 81/08 were limited to assessing occupational risks and conducting health surveillance of exposed workers. The relationship with the general practitioners (GPs) was left entirely to individual initiative, with predictable results: Persechino et al. [1] found that cooperation was perceived as difficult by most of the Italian OPs interviewed, hindered by a lack of time, the absence of communication channels, and unclear role boundaries.

Law 198/2025 (converting Decree-Law 159/2025), which entered into force on December 31, 2025, changes this landscape. New Article 25, Paragraph 1, Letter *a-bis* of Legislative Decree 81/08 requires occupational physicians to inform workers about the importance of cancer prevention and to promote participation in National Health Service (SSN) screening programs included in the Essential Levels of Care (LEA) [2]. For the first time, the workplace is legally recognized as the ideal setting for fulfilling public health functions that extend beyond occupational risk. This expanded mandate is difficult to fulfill without a structural link between

the OP and the GP, who typically coordinates the patient's preventive care and holds information on screening participation. Without access to this information, the OP either duplicates the GP's work or acts blindly.

Two further legislative developments reinforce the need for integration. Legislative Decree 135/2024, which transposes EU Directive 2022/431, expands regulations on carcinogenic, mutagenic, and reprotoxic substances and emphasizes the need for post-exposure follow-up, which inevitably involves the GP once workplace monitoring has ended. Legislative Decree 29/2024 frames the employer's existing obligations under Legislative Decree 81/08 within a broader Workplace Health Promotion (WHP) model, referencing the WHO's approach to workplace health promotion and the National Prevention Plan (PNP) [4]. Although this does not create new obligations beyond those already established by Article 28 of Legislative Decree 81/08, it signals a policy expectation that occupational health should address population aging and chronic diseases—an expectation that presupposes coordination with primary care.

The policy context is further defined by the 2020-2025 National Prevention Plan (PNP), adopted through the State-Regions Agreement of August 6, 2020 [28], which serves as the central planning instrument for prevention and health promotion in Italy. The PNP adopts a *life-course* and *setting-based* approach and explicitly identifies the workplace as a privileged setting for health promotion interventions, citing the Total Worker Health® (TWH®) model [3, 26] as a reference framework for corporate health initiatives. This vision aligns with the ICOH International Code of Ethics, which mandates that occupational health professionals safeguard and promote workers' health and working capacity [27]. Complementing the PNP is the National Chronic Disease Plan (PNC), updated by the State-Regions Agreement of October 23, 2025 [29], which identifies prevention as a key element of chronic disease management and refers to PNP interventions to reduce modifiable risk factors. Taken together, the PNP and PNC establish a policy architecture in which the workplace is expected to contribute to population health goals, a contribution that, in

operational terms, requires the OP and the GP to share information on the same patient.

A decisive step forward has been taken with the adoption of the new National Prevention Plan (PNP) 2026-2031 [30] through the State-Regions Agreement of May 21, 2026. The new PNP identifies seven priority intervention areas, including "Health, safety and well-being of workers". Building on the previous Plan but with greater operational ambition, the PNP 2026-2031 promotes the Italian Total Worker Health (ITWH) model as an integrated set of policies, programs, and practices that combine the prevention of occupational health and safety risks with actions to prevent acute and chronic harm, in pursuit of broader worker well-being. The new Plan reinforces the workplace as a privileged setting for health promotion and calls for optimizing integration among prevention services, primary care, territorial assistance, hospitals, and rehabilitation. This explicit reference to an integrated care pathway across settings provides the policy foundation for the OP-GP collaboration model proposed in our roadmap. Furthermore, the PNP 2026-2031 adopts an Evidence-Based Prevention (EBP) approach and mandates the development of Regional Prevention Plans by November 30, 2026, creating a regulatory window within which pilot OP-GP collaboration programs could be embedded at the regional level.

On the same date, the State-Regions Conference also approved, through a formal agreement, the first National Strategy for Occupational Health and Safety 2026-2030 [31]. This Strategy, aligned with the EU Strategic Framework 2021-2027 [5] and explicitly integrated with the PNP 2026-2031, is built on five strategic axes: addressing the transformations of work (digitalization, AI, ecological transition, new contractual forms); strengthening the resilience of the institutional system; enhancing the effectiveness of protections; supporting micro, small and medium-sized enterprises; and disseminating a culture of prevention from schools onward. The Strategy adopts the Vision Zero approach, introduced in Italy for the first time, according to which every workplace incident is preventable. Notably, the Strategy calls for strengthened coordination among supervisory bodies (Local Health Agencies,

National Labor Inspectorate) and more targeted inspections, and its integration with the PNP creates a dual policy framework in which corporate TWH programs are no longer optional add-ons but embedded components of the national prevention architecture.

2. FOUR ARGUMENTS IN FAVOR OF STRUCTURED COLLABORATION

2.1. Continuity of Post-Exposure Care

When a worker exposed to CMR agents (carcinogenic, mutagenic or toxic for reproduction) retires or changes jobs, occupational health surveillance typically ceases, even though the disease's latency period continues, often for decades. The GP taking over the patient's care should be aware of the nature, duration, and intensity of previous occupational exposures to ensure adequate secondary prevention. This requirement is in line with the EU Strategic Framework for Occupational Safety and Health 2021-2027 [5] and the European Cancer Plan, both of which focus on lifelong prevention. In practice, however, this transfer of information is left to the worker's discretion, who may or may not provide the GP with a copy of the individual health and risk record or even the assessment issued by the OP following the end-of-employment medical examination. In practice, this rarely occurs, but it would be more logical for information to be transferred directly between the two healthcare professionals.

2.2. Coordination of Cancer Screening

The new Article 25 *a-bis* creates a practical coordination challenge. If the OP promotes cancer screening without knowing whether the worker has already been screened through the National Health Service (SSN), the intervention is redundant or misdirected. Conversely, the workplace offers a unique opportunity for access: workers who do not visit the GP office are nonetheless examined periodically by the occupational physician as part of health surveillance. The OP can reinforce GP recommendations, but only if both share basic information on the patient's preventive health status. As argued by Buijs et al. [6],

patients often trust their GP more than the OP, making combined efforts more effective than isolated interventions.

2.3. Fitness for Work and Drug Interference

This is likely the most compelling yet least discussed argument in favor of collaboration between OP and GP. The OP *fitness-for-duty* assessment is designed to protect the health and safety of the individual worker and third parties, including colleagues and the public, when the job involves safety risks. The accuracy of the assessment depends on a comprehensive understanding of the worker's health status, including current medication regimens. Commonly used medications—sedatives, first-generation antihistamines, opioid analgesics, psychotropic drugs, and certain hypoglycemic agents—can impair alertness, reaction times, and decision-making. A worker who operates heavy machinery, drives commercial vehicles, or works at heights while taking such medications poses a significant safety risk.

Workers may conceal this information from the OP for understandable reasons: fear of losing their job, stigma, or simply because they do not recognize the professional relevance of their prescriptions. The GP, who manages the patient's medication history over time, holds the missing information. Targeted sharing of clinically relevant data—strictly limited to what affects workplace safety—could enable the OP to make truly protective fitness-for-work assessments and prevent workplace accidents that current information asymmetries leave unnoticed.

2.4. Foreign-Born Workers: Endemic Diseases and Language Barriers

A further dimension of the OP-GP collaboration gap concerns Italy's growing population of foreign-born workers. As of January 2024, approximately 5.3 million foreign residents, or 10% of the total population, are disproportionately represented in physically demanding and hazardous occupations—construction, agriculture, logistics, metalworking, and domestic care [7]. INAIL (National Institute for Insurance against Accidents at Work) data for 2025 show that injuries to foreign-born workers

now account for 24.3% of all workplace injury reports, up from 23.5% in 2024, with a year-on-year increase of 3.7%, compared with a decrease of 0.5% among Italian-born workers. Fatal injuries in the foreign-born group also rose, from 176 to 182 reports [7].

These workers often come from countries where tuberculosis, hepatitis B and C, strongyloidiasis, schistosomiasis, and other parasitic infections are endemic. The ECDC evidence-based guidance on infectious disease screening in newly arrived migrants recommends voluntary screening for active and latent TB, HIV, HBV, HCV, strongyloidiasis, and schistosomiasis [8], while the OMEGA-NET position paper highlights that preventive services for infectious diseases, mental health, and chronic conditions are the most neglected areas in occupational health provision for migrants [9]. When these workers enter the Italian labor market, neither the OP nor the GP may be aware of pre-existing conditions that are clinically silent yet relevant to fitness evaluation—latent TB in a worker assigned to immunosuppressive dust environments, chronic HBV carriage in a healthcare auxiliary, or untreated parasitic infections that may interact with occupational chemical exposures.

The problem is compounded by language barriers. Workers with limited proficiency in Italian may be unable to communicate symptoms accurately to the OP during health surveillance, to understand the meaning of a fitness judgment with prescriptions or limitations, or to relay the OP's recommendations to their GP. Conversely, the GP—when a foreign-born worker is registered with the SSN—may prescribe treatments without knowledge of workplace exposures. The ILO has identified language and cultural barriers, together with precarious employment conditions, as key determinants of occupational health disparities among migrant workers [10]. The WHO Refugee and Migrant Health Toolkit explicitly recommends integrated health assessments that bridge occupational and primary care for this population [11].

For these workers, OP-GP data sharing is not merely useful but often indispensable. The GP may hold results from initial health assessments conducted upon entry, vaccination records from the country of origin, and diagnoses of endemic

conditions. The OP needs this information to issue an informed fitness judgment. Conversely, the OP's knowledge of specific workplace exposures—silica dust, welding fumes, pesticides, solvents—is critical for the GP managing a patient whose baseline health profile already includes vulnerabilities. The consent-based exchange model proposed in this roadmap (Section 5, Level 1) should include multilingual consent forms and, where feasible, cultural mediation support to ensure that the worker's informed consent is genuine.

3. INTERNATIONAL EXPERIENCE: PERSISTENT DIFFICULTIES

The Italian situation is not unique in Europe: collaboration between occupational physicians and general practitioners has consistently proven difficult, and an honest assessment of the situation at the international level is necessary.

1. In Germany, Stratil et al. [12, 13] conducted focus groups with GPs, OPs, and rehabilitation physicians, identifying organizational barriers (lack of contact information, conflicting schedules), interpersonal obstacles (low trust in occupational physicians, perceived conflicts of interest with employers), and structural constraints (data privacy regulations). The proposed solutions included formal involvement of occupational physicians in rehabilitation, financial incentives for cooperation, and joint training programs, most of which have not yet been implemented years later.
2. In France, Verger et al. [14] surveyed national samples of GPs and OPs and found that although most endorsed collaboration, a significant proportion of GPs did not trust OPs' independence, a perception that inhibited referrals and data sharing. Moßhammer et al. [15] reported similar findings in Baden-Württemberg: telephone contact was the predominant mode of communication (used by 91% of OPs but only 49% of GPs), and GPs consistently underestimated the need for cooperation.

3. In Finland, primary care is organized into three parallel sectors—public, private, and occupational—all accessible to the working population. Reho et al. [16] found that patients who use multiple healthcare sectors in parallel face a significantly higher risk of disability retirement (OR 4.53), underscoring the clinical cost of fragmented care. Savinainen et al. [17] interviewed healthcare professionals in small Finnish municipalities and identified a lack of familiarity between OPs and GPs, inadequate information exchange, and insufficient time as the main barriers to cooperation.
4. In the Netherlands, interprofessional collaboration between the OP and the GP has been a policy goal for decades, yet Vossen et al. [18] recently documented that it remains largely unfulfilled. They assessed whether delegating collaborative tasks to assistant practitioners and practice nurses could overcome some barriers, but found that even these intermediary professionals still face difficulties in cross-sectoral communication.

The lesson from these experiences is that the value of collaboration between OPs and GPs is universally recognized, yet it has not been systematically achieved through calls for collaboration alone. Structural and regulatory interventions are needed.

4. OBSTACLES IN THE ITALIAN CONTEXT

The specific structural characteristics of Italy present additional challenges that the three-tiered strategy outlined below must address.

- *The professional fragmentation of OPs.* Many Italian OPs work as independent professionals serving multiple companies simultaneously, often across different provinces. This model makes it difficult to establish stable relationships with the GPs of workers at a single company. Unlike integrated models (e.g., Finnish occupational health centers), the Italian system lacks a physical or institutional setting where the OP and the GP can interact naturally.
 - *The FSE 2.0: Promises and Reality.* The Electronic Health Record 2.0 (*Fascicolo Sanitario Elettronico* 2.0) appears, in principle, to be the most appropriate infrastructural solution. However, the results of its implementation call for caution. Monitoring by the GIMBE Foundation in early 2026 revealed that no region had made all 20 required document types available; only 44% of citizens had consented to healthcare providers accessing their data; and significant interregional disparities persist, with the southern regions lagging considerably behind [19]. The Ministerial monitoring portal confirms that the Phase 3 deadline of March 31, 2026, was reached with substantial gaps in operational readiness across regions [20]. The *Profilo Sanitario Sintetico* (Patient Summary), which GPs are required to complete, has been criticized by the SNAMI medical union for the burden of manual compilation and for unclear medico-legal implications [21]. A section dedicated to occupational medicine within the FSE—the roadmap’s long-term objective—is not currently provided for in any regulatory instrument and would require specific legislative action.
 - *Privacy and Liability.* The OP serves as an independent data controller under the GDPR for health data collected during occupational health surveillance. Any sharing of data with GPs must comply with the principle of data minimization: only information strictly necessary for continuity of care, secondary prevention, or an accurate assessment of fitness may be exchanged. The employer must not
- *The workload and incentive structure of GPs.* Italian GPs operate under National Collective Agreements, typically managing up to 1,500 patients each, with per-capita remuneration. Their time for patient visits is severely limited. Adding structured communication with OPs for their active patients requires dedicated time, financial compensation, or both. Without such incentives, the burden on GPs risks becoming yet another unfunded obligation.

have access to clinical diagnoses or pharmacological data shared between the OP and the GP; the employer is entitled only to the final assessment of fitness. An unresolved issue concerns professional liability: if a GP shares pharmacological information with the OP and the OP nevertheless issues an inaccurate fitness-for-work assessment, or conversely if the GP omits relevant information, the attribution of liability remains poorly defined in Italian case law.

5. A ROADMAP FOR THREE-TIERED IMPLEMENTATION

Based on international evidence and the specific characteristics of the Italian system, we propose a phased strategy ranging from immediate feasibility to structural integration (Figure 1).

- *Level 1: Consent-based exchange (immediate).* During routine occupational health visits, the OP asks the worker to sign a GDPR-compliant consent form authorizing the two-way sharing of clinically relevant information

with their GP. The shared data is limited to a standardized minimum set: current occupational exposures, fitness limitations and prescriptions, screening adherence status (from the general practitioner to the occupational physician), and relevant pharmacological therapies affecting workplace safety (from the general practitioner to the occupational physician). The worker serves as an intermediary and retains full control. For foreign-born workers, consent forms should be available in the most commonly spoken languages, and, where feasible, the exchange should be supported by cultural mediation services. This model requires no regulatory changes and can be implemented immediately.

- *Level 2: InterSociety Protocols (medium term).* Scientific societies representing occupational medicine and general practice should jointly develop standardized communication protocols that define: the minimum data set content, secure transmission procedures, pseudonymization requirements, and clinical scenarios that trigger mandatory information exchange (e.g., cessation of exposure to CMR substances, a new prescription of

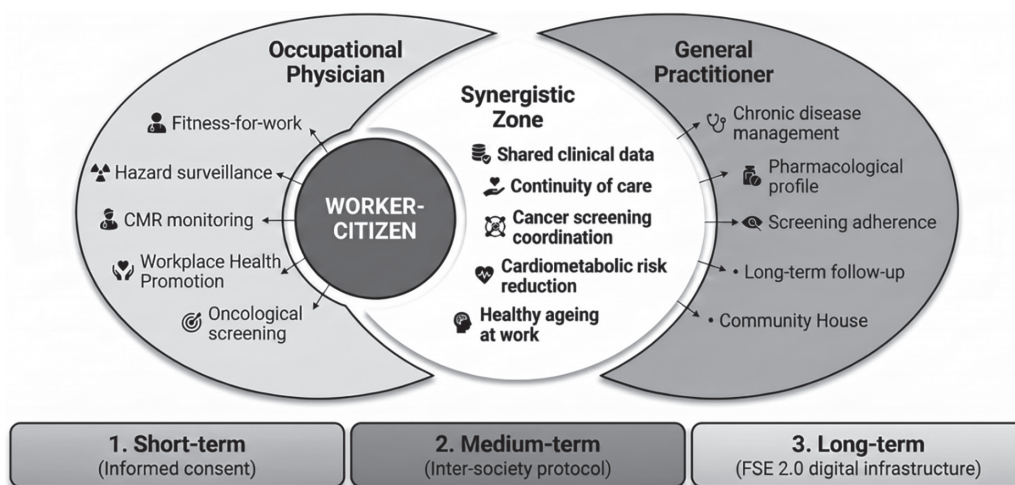


Figure 1. Conceptual framework for the synergistic collaboration between the Occupational Physician (OP) and the General Practitioner (GP) within a Total Worker Health® (TWH) approach, highlighting shared preventive domains (Synergistic Zone) and a three-tiered implementation strategy. CMR: carcinogen, mutagen, reprotoxic agents; WHP: Workplace Health Promotion; FSE 2.0: Fascicolo Sanitario Elettronico 2.0 (Electronic Health Record).

sedative medications for a worker performing safety-critical tasks). Pilot implementation could be hosted within the Community Centres (*Case della Comunità*) established under Ministerial Decree 77/2022 and funded by the PNRR, which provide physical locations for multidisciplinary healthcare teams. As documented by Leso et al. [22], Italian occupational physicians identify structured interdisciplinary collaboration and targeted training in WHP as their primary professional needs.

- *Level 3: Legislative and Digital Integration (long-term).* The goal is a systemic communication channel that does not depend on individual initiative. This requires two coordinated interventions: first, an amendment to Article 25 of Legislative Decree 81/08 mandating the sharing of clinically relevant health data between OP and GP for documented purposes (continuity of post-exposure care, accuracy of fitness assessments, coordination of screening); second, the creation of a role-based section dedicated to occupational health within the FSE 2.0. Given the current gaps in implementing the FSE [19-21], this level is realistic only once the basic infrastructure achieves reliable national coverage—a goal currently tentatively set for 2026 but likely to extend beyond that date.

6. EVIDENCE FROM WORKPLACE HEALTH PROMOTION

The clinical rationale for integrating occupational medicine and general practice is supported by recent evidence on the effectiveness of health promotion in occupational settings. The three-level meta-analysis by Godono et al. [23], which includes 44 studies and nearly 50,000 participants, demonstrated significant reductions in body mass index, blood pressure, and LDL cholesterol through workplace health promotion interventions. The effect was most pronounced when programs targeted high-risk populations, were led by qualified healthcare professionals, and were integrated into sustainable health promotion policies. Health promotion initiatives are precisely

those that benefit from coordination between OPs and GPs: the OP identifies at-risk workers during surveillance, and the GP provides the clinical context and capacity for follow-up.

Magnavita et al. [24] and Talini and Baldasseroni [25] have emphasized that workplace health promotion activities in Italy require ongoing institutional commitment and must not replace mandatory health and safety measures. Health promotion programs require adequate follow-up to measure actual health benefits—a requirement that, once again, implies continuity between occupational medicine and primary care settings.

7. CONCLUSION

The convergence of Law 198/2025, Legislative Decree 135/2024, and Legislative Decree 29/2024 creates a legislative framework in which collaboration between occupational medicine and general practice is no longer optional but a prerequisite for legal compliance. However, legislation alone does not produce collaboration, as decades of experience in the Netherlands, Germany, France, and Finland demonstrate. The obstacles are structural: incompatible professional incentives, a lack of communication infrastructure, privacy constraints, and unresolved liability issues. The adoption of the PNP 2026-2031 and the National Strategy for Occupational Health and Safety 2026-2030 [30,31], both approved on May 21, 2026, reinforces this framework by explicitly embedding the TWH model within the national prevention architecture and mandating the integration of prevention services with primary care and territorial assistance. This policy direction is fully aligned with the roadmap proposed here.

The three-phase roadmap proposed here addresses these obstacles incrementally, from immediately feasible consensus-based exchanges to the long-term integration of occupational health data into the FSE 2.0. Its success depends on the commitment of scientific societies to develop shared protocols, of regional health authorities to create pilot programs within Community Health Centers, and of the legislature to extend Article 25 of Legislative Decree 81/08 to make bidirectional data sharing mandatory for defined clinical purposes.

A reform of the legal status of general practitioners is currently under consideration in Italy: whether they should remain self-employed practitioners under the *SSN* (with new rules) or, on a voluntary basis, switch to the status of civil servants within the *SSN*. In the proposals under discussion, references to 'prevention' and 'early diagnosis' of cancer are generic and geared toward proactive care for the chronically ill and frail, not toward post-exposure occupational health surveillance as per Articles 242 and 259 of Legislative Decree 81/08. The legislation should ideally be amended accordingly.

The worker and the citizen are one and the same person. Their health cannot be effectively managed by two professionals who do not communicate.

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